INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Talazoparib · breast monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00605
Version 2

Daily continuous treatment

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Control advanced cancer and symptoms while balancing side effects and treatment burden.

ORAL PARP INHIBITOR

Talazoparib

Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) repair in BRCA-associated cancer

PARP inhibited / trapped → DNA damage accumulates → BRCA-deficient cells less able to repair

Talazoparib inhibits PARP and traps it on damaged DNA. Cancer cells with inherited BRCA1/2 changes may have less ability to repair this damage. Normal bone marrow can also be affected, so blood-count monitoring is essential.

Report fever, infections, unusual bruising, persistent fatigue or breathlessness.

Why this treatment?

Talazoparib targets a DNA-repair vulnerability associated with inherited BRCA1/2 changes. Suitability also depends on HER2 status, previous anthracycline/taxane treatment and, for hormone-receptor-positive disease, previous endocrine treatment or its suitability.

Your treatment setting

Germline BRCA1/2-mutated, HER2-negative locally advanced or metastatic breast cancer

NCCP 00605 v2 · official regimen

Your exact schedule

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

NCCP 00605 v2 · official regimen

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

NCCP 00605 v2 · official regimen

Why this treatment may be chosen

Discuss your own expected benefit, alternatives and uncertainty with the treating team.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00605 v2 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Talazoparib · breast monotherapy: treatment and checks

Your treatment pattern

Daily continuous treatment

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00605	gBRCA / HER2– eligibility	Daily capsules start	Daily without planned breaks	Until progression / intolerance	Next treatment plan
Blood / organ function NCCP 00605	full blood count (FBC), renal, liver	Early toxicity review	FBC / renal / liver every 4 weeks	Counts and restart review	Investigate prolonged cytopenia
Symptoms / medicines NCCP 00605	Baseline assessment	Early symptom review	Keep the individual check plan	Reconcile at treatment changes	Report continuing symptoms
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 **Before treatment**

Before the first dose

Germline BRCA1/2-mutated, HER2-negative locally advanced or metastatic breast cancer. Check FBC, kidney and liver function before starting and every four weeks. More frequent blood review may be needed for low counts; prolonged abnormalities require specialist assessment.

Ask / **act:** Confirm eligibility, test results and medicine list.

2 **Other checks**

At the specified review points

Confirm the germline BRCA result and previous treatment. Review pregnancy status and contraception, kidney function and interacting medicines, particularly P-glycoprotein inhibitors.

Ask / **act:** Confirm who arranges and reviews each check.

3 **Support and practical plan**

At each treatment or phase change

Blood monitoring, adherence, prompt infection/bleeding review and individually prescribed rescue antiemetics.

Ask / **act:** Choose the matching support plan; confirm active medicines.

4 **Response and follow-up**

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / **act:** Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

Check FBC, kidney and liver function before starting and every four weeks. More frequent blood review may be needed for low counts; prolonged abnormalities require specialist assessment.

NCCP 00605 v2 · official regimen

Other treatment-specific checks

Confirm the germline BRCA result and previous treatment. Review pregnancy status and contraception, kidney function and interacting medicines, particularly P-glycoprotein inhibitors.

NCCP 00605 v2 · official regimen

Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

ESMO metastatic breast cancer guideline · 2026 · ASCO · biomarkers in metastatic breast cancer

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

NICE NG101 · bone health and follow-up

The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

Treatment changes

Do not replace a missed or vomited dose. Take the next dose at the usual time. The team decides whether and at what dose to restart after a hold.

NCCP 00605 v2 · official regimen

Medicines review

Ask the pharmacist to review P-glycoprotein inhibitors and all new medicines or supplements. Changing kidney function or an interacting medicine can change the prescribed dose.

NCCP 00605 v2 · official regimen

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

NCCP 00605 v2 · official regimen

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily treatment and four-weekly reviews

No dates are entered. Columns show milestones, not equal time intervals; follow your individual prescription.

Treatment or check	Timing in this view
Before starting	**Before starting** Eligibility, tests and medicines review
Treatment	**First weeks → Four-weekly review → Ongoing treatment** Daily capsules without a scheduled rest week
Monitoring / support	**First weeks → Four-weekly review → Ongoing treatment → Next decision** Keep the treatment-specific test and symptom plan
Next decision	**Next decision** Confirm continuation or completion

Follow the individual prescription; dose changes are made by the treating team.

Review blood counts and organ function every four weeks or more often when needed.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Blood counts

Keep four-weekly blood reviews and any additional tests. Fever or unusual bleeding needs prompt advice.

Taking capsules

Use the strength and number on your current prescription. Do not replace a missed or vomited dose.

Nausea

Use the prescribed rescue plan; call if vomiting affects fluids or tablets.

Medicines review

Check every new medicine or supplement with the oncology pharmacist.

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

Swallow capsules whole with water, with or without food.

Do not open or dissolve them.

A daily routine

Choose a consistent daily time.

Arrange supplies before they run out.

Reviews without routine infusion visits

No port or infusion pump is needed for talazoparib itself.

After treatment

Report persistent unexplained fatigue, infections or bruising; rare bone-marrow disorders can require investigation.

Questions to ask

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Explain the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Talazoparib

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic premedication is required. Select a PRN antiemetic under the oral Table 5/local plan; review renal function, QT risk and interactions.
- Monitor blood counts and organ function every four weeks; transfusion or growth-factor support is individualised.
- Review capsule strength, adherence and interacting medicines before issuing a new prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Talazoparib · breast monotherapy

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

Oral PARP inhibitor · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood / infection

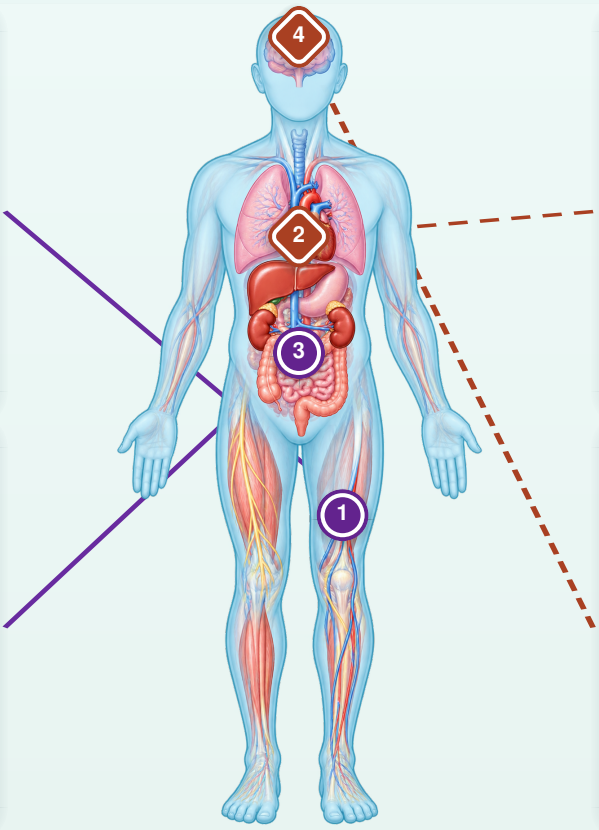
Talazoparib

Fever, infection or unusual bruising needs prompt advice.

2 Energy / breathing

Talazoparib

Marked fatigue or breathlessness may reflect anaemia.



3 Nausea / appetite

Talazoparib

Call if vomiting or poor intake prevents fluids or medicines.

4 Hair / scalp

Talazoparib

Hair thinning or loss can occur; ask about practical support.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Fatigue / breathlessness	☐	☐	☐	
Infections / bruising	☐	☐	☐	
Nausea / vomiting	☐	☐	☐	
Appetite changes	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Talazoparib · breast monotherapy

NCCP 00605 v2

Talazoparib is taken once daily continuously until disease progression or unacceptable toxicity. The usual starting dose is 1 mg; kidney function, interactions and side effects can require a different prescription.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact oncology urgently for fever, shivers, infection, unusual bleeding, marked fatigue or new breathlessness.

https://sactcheck.com/patient/00605/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.19 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00605 v2 · official regimen**
   https://healthservice.hse.ie/documents/6414/605_v2_Talazoparib_Therapy.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
9. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
10. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **NCCP oral antiemetic classification v6 · Tables 5 and 8**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
13. **EMA Talzenna product information**
    https://www.ema.europa.eu/en/documents/product-information/talzenna-epar-product-information_en.pdf